PANACID TABLET

DESCRIPTION:
A yellow to orange coloured, oblong shaped film coated tablet with a score on one side.

COMPOSITION:
Each film coated tablet contains Mefenamic Acid 500mg.

ACTIONS & PHARMACOLOGY:
Mefenamic acid is a non-steroidal analgesic, anti-inflammatory and antipyretic agent. It inhibits the activity of the enzyme cyclo-oxygenase, resulting in decreased formation of precursors of prostaglandins and thromboxanes from arachidonic acid. It also inhibits completely the actions of prostaglandins. The resultant decrease in prostaglandin synthesis and activity in various tissues is responsible for many of the therapeutic and adverse effects of mefenamic acid. Its antidysmenorrheal action is due to the inhibition in the synthesis and activity of intrauterine prostaglandins as well as due to decreased uterine contractility & uterine pressure, increase uterine perfusion, and relieve ischemic as well as spasmodic pain. Mefenamic acid is absorbed from the gastrointestinal tract after oral administration. Following a single 1 gram oral dose, peak plasma levels of 10 mcg/ml occurred in 2 to 4 hours. Following multiple doses, plasma levels are proportional to dose with no evidence of drug accumulation. Mefenamic acid is extensively bound to plasma proteins. Biotransformation is hepatic. The half-life is reported to be 2 to 4 hours. Following a single dose, sixty seven percent of the total dose is excreted in the urine as unchanged drug or conjugated metabolites. Twenty to twenty five percent of the dose is excreted in the faeces during the first three days.

INDICATIONS:
As an anti-inflammatory analgesic for the symptomatic relief of rheumatoid arthritis, osteoarthritis and pain including muscular, traumatic and dental pain, headaches of most aetiology, post- operative and post-partum pain. Symptomatic relief of primary dysmenorrhoea.

CONTRAINDICATIONS:
Should not be used in patients who have previously exhibited hypersensitivity to it. Also contraindicated in patients with active ulceration or chronic inflammation of either the upper or lower gastrointestinal tract. Should not be given to patients with renal or hepatic function impairment. Should not be given to patients with pre-existing hypoprothrombinemia and the concurrent use with any drug which causes hypoprothrombinemia is also contraindicated. Because the potential exists for cross-sensitivity to aspirin or other nonsteroidal anti-inflammatory drugs, mefenamic acid should not be given to patients in whom these drugs induce symptoms of bronchospasm, allergic rhinitis, or urticaria.

DRUG INTERACTIONS:
Mefenamic acid may prolong prothrombin time. When the drug is administered to patients receiving oral anticoagulant drugs, monitoring of prothrombin time as clinically indicated is necessary.

SIDE EFFECTS:
The most frequently reported adverse reactions associated with the use of mefenamic acid are gastrointestinal disturbances including diarrhoea, nausea with or without vomiting and abdominal pain. Peptic ulceration and gastrointestinal bleeding have also been reported. There may be hypersensitivity reactions including skin rashes and urticaria. Rare incidence of drowsiness, dizziness, nervousness, headache, blurred vision, insomnia, eye irritation, ear pain,

papillary necrosis, hematuria, dysuria, leukopenia, eosinophillia, thrombocytopenic purpura, agranulocytosis, pancytopenia and bone marrow hypoplasia have also been reported.

Skin and subcutaneous tissue disorders:
Generalised bullous fixed drug eruption (GBFDE)

WARNING AND PRECAUTIONS:

Risk of GI Ulceration, Bleeding and Perforation with NSAID: Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (eg, dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms. In patients observed in clinical trials of several months to 2 years duration, symptomatic upper GI ulcers, gross bleeding or perforation occurred in approximately 1% of patients treated for 3 to 6 months, and in about 2% to 4% of patients treated for 1 year. Higher percentages have been reported by other independent studies. Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding.

Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (eg, alcoholism, smoking, corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events. Measures such as the use of physical therapy and mild analgesics like paracetamol (when inflammation is not a major factor) should be instituted prior to initiation of therapy with NSAID. NSAIDs should only be used after proper appraisal of potential risks to patients. It should be used with the lowest effective dose for only as long as needed. This drug should not be co-administered with other NSAIDs. Prescribers should inform patients about the signs and/or symptoms of serious GI toxicity and what steps to take if they occur. In considering the use of relatively large doses (within the recommended dosage range), sufficient benefit should offset the potential increased risk of GI toxicity.

It is recommended that mefenamic acid therapy be discontinued promptly if diarrhoea or skin rash develops and should not be used for more than 7 days is not recommended. Patients who develop diarrhoea during therapy are usually unable to tolerate the drug thereafter. Mefenamic acid should be administered immediately after meals or with food or antacids to reduce gastrointestinal irritation. Caution should be exercised when other nonsteroidal anti-inflammatory drugs, anticoagulants or thrombolytic agents, antidiabetic agents & anti-hypertensive drugs are concurrently used. Caution should be exercised in patients who require surgery. Elderly patients are more likely to have age-related renal function impairment which may increase the risk of NSAID – induced hepatic or renal toxicity and may also require dosage reduction to prevent accumulation of the drug. Also NSAID – induced gastrointestinal ulceration and/or bleeding may be more likely to cause serious consequences, including fatalities, in geriatric patients than in younger adults. NSAID including mefenamic acid may cause soreness, irritation or ulceration of the oral mucosa. A false-positive test may occur when diazo tablet test is used to determine the urinary bile presence. Prothrombin time may also be prolonged. The antipyretic, analgesic and anti-inflammatory actions of NSAID may mask symptoms of the occurrence or worsening of infections.

Cardiovascular Thrombotic Events
Observational studies have indicated that non-selective NSAIDs may be associated with an increased risk of serious cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease or cardiovascular risk of an adverse cardiovascular event in patient taking NSAID, especially in those with cardiovascular risk factors, the lowest effective dose should be used for the shortest possible duration.
There is no consistent evidence that the concurrent use of aspirin mitigates the possible increased risk of serious cardiovascular thrombotic events associated with NSAID use

Hypertension
NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking antihypertensive with NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAID treatment and at regular intervals thereafter.

Heart Failure
Fluid retention and oedema have been observed in some patients taking NSAIDs, there caution is advised in patients with fluid retention or heart failure.

Gastrointestinal Events
All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulcerations occur in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patients about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of aspirin and NSAIDs also increases the risk of serious gastrointestinal adverse events.

Severe Skin Reactions
NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reactions and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.

Serious skin reactions such as Generalised bullous fixed drug eruption (GBFDE) have been reported very rarely in association with the use of mefenamic acid. Mefenamic acid should be discontinued at the first appearance of the skin rash, mucosal lesions or any other sign of hypersensitivity.

PREGNANCY & LACTATION:
Use of mefenamic acid in pregnancy and to nursing mothers is not recommended.

DOSAGE & ADMINISTRATION:
Adult, children over 14 years of age and geriatric: 500mg three times daily. Dosage may be reduced to 250mg four or three times daily in the occasional patient unable to tolerate the 500mg dosage level. In dysmenorrhoea, administration should start with onset of menses and continue for five days or until discomfort or flow ceases, whichever is less. Take preferably with food or immediately after meals.

After assessing the risk/benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used.

ROUTE OF ADMINISTRATION:
Oral administration

EFFECTS ON ABILITY TO DRIVE AND USE MACHINE:
Mefenamic Acid may cause drowsiness, dizziness, fatigue or affect your vision. If any of these occur do not drive, use machinery or perform any tasks that may require you to be alert.

OVERDOSAGE & TREATMENT:
Symptoms: Symptoms of gastrointestinal, renal and central nervous system toxicities which include abdominal pain, nausea, vomiting, gastrointestinal hemorrhage, acute renal failure, convulsions, hypoprothrombinemia etc.
Treatment: The stomach should be emptied by inducing emesis or by careful gastric ravage followed by repeated administration of activated charcoal. Vital functions should be monitored and supported. Institute symptomatic and other supportive treatment as necessary.

PRESENTATION:
Blisters of 10 tablets. Box of 100, 500 or 1000 tablets.

STORAGE CONDITIONS:
Store in a cool dry place below 30°C.
Keep medicines out of reach of children.
Protect from light.

MANUFACTURED BY:
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